

**REFERRAL COVER SHEET**  
Fax Transmission - Medical Records

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|                                   |                                             |
|-----------------------------------|---------------------------------------------|
| <b>TO:</b>                        | Johns Hopkins Hospital - Dept. of Neurology |
| <b>Attn:</b>                      | Neurology Referral Intake / Scheduling      |
| <b>Fax #:</b>                     | (410) 955-XXXX                              |
| <b>FROM:</b>                      | Riverside Internal Medicine Associates      |
| <b>Referring Provider:</b>        | Dr. Patricia Ngozi, MD (NPI: 1982734056)    |
| <b>Fax:</b>                       | (571) 240-8812                              |
| <b>Phone:</b>                     | (571) 240-8800                              |
| <b>Date of Transmission:</b>      | 03/14/2024 10:47 AM                         |
| <b>Total Pages (incl. cover):</b> | 18                                          |

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**PATIENT:**

Margaret A. Kowalski    DOB: 06/12/1978    MRN (ours): 04-389-2271

**REASON FOR REFERRAL:**

Patient presenting with progressive sensory sx and intermittent visual complaints.  
Requesting neuro eval - ? etiology. Please see attached notes.

**URGENCY:**

Routine / Non-urgent (per referring provider)

**INSURANCE:**

BlueCross BlueShield Federal Employee Program  
Member ID: XYZ884129201    Group: FEP001

*NOTE: Some pages may be out of order due to EHR system export.  
Please disregard duplicate labs - system error on export 03/13.*

*CONFIDENTIALITY NOTICE: This facsimile contains privileged and confidential  
information intended only for the use of the individual named above. If you  
have received this fax in error, please notify sender immediately.*

**RIVERSIDE INTERNAL MEDICINE ASSOCIATES**

8821 Old Courthouse Rd, Suite 110 | Vienna, VA 22182

Tel: (571) 240-8800 Fax: (571) 240-8812

**PATIENT DEMOGRAPHIC INFORMATION**

|                           |                                             |
|---------------------------|---------------------------------------------|
| <b>Full Name:</b>         | Kowalski, Margaret Ann                      |
| <b>Preferred Name:</b>    | Maggie                                      |
| <b>Date of Birth:</b>     | June 12, 1978 (Age: 45)                     |
| <b>Sex:</b>               | Female                                      |
| <b>Race/Ethnicity:</b>    | White / Non-Hispanic                        |
| <b>SSN (last 4):</b>      | ****-**-6614                                |
| <b>MRN (Riverside):</b>   | 04-389-2271                                 |
| <b>Address:</b>           | 412 Burnside Mill Ct, Centreville, VA 20120 |
| <b>Phone (home):</b>      | (703) 555-0184                              |
| <b>Phone (cell):</b>      | (703) 555-9927                              |
| <b>Emergency Contact:</b> | David Kowalski (husband) (703) 555-9928     |
| <b>Occupation:</b>        | High School Biology Teacher                 |
| <b>Employer:</b>          | Fairfax County Public Schools               |

**INSURANCE / BILLING:**

|                           |                                     |
|---------------------------|-------------------------------------|
| <b>Primary Insurance:</b> | BCBS Federal Employee Program (FEP) |
| <b>Member ID:</b>         | XYZ884129201                        |
| <b>Group:</b>             | FEP001                              |
| <b>Policy Holder:</b>     | Margaret A. Kowalski (self)         |
| <b>Copay:</b>             | \$30 specialist / \$15 PCP          |
| <b>Auth Required:</b>     | YES - Auth pending as of 03/13/2024 |
| <b>Secondary:</b>         | None                                |

**PHARMACY:**

CVS Pharmacy #4421 | 14122 Lee Hwy, Centreville VA | (703) 830-2200

PCP on file: Dr. Patricia Ngozi, MD (Riverside IM)

Last PCP visit: 02/28/2024

**RIVERSIDE INTERNAL MEDICINE ASSOCIATES**  
**REFERRAL LETTER / PROVIDER NOTE**

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**DATE: 02/28/2024**

PATIENT: Kowalski, Margaret A.    DOB: 06/12/1978

PROVIDER: Patricia Ngozi, MD

VISIT TYPE: Office Visit - Established Patient

REFERRING TO: Neurology - Johns Hopkins Hospital

Dear Neurology Colleague,

I am writing to refer my patient, Ms. Margaret Kowalski, a 45-year-old female school teacher

with no significant past neurological history, for evaluation of progressive neurological

symptoms that have developed over the past 8-10 months. I believe subspecialty evaluation

is warranted and would appreciate your assessment.

**CHIEF COMPLAINT / REASON FOR REFERRAL:**

Patient presents with two distinct episodes of neurological symptoms separated in time.

The first episode occurred approximately 10 months ago; the second began 6 weeks ago.

Symptoms include sensory disturbances (numbness, tingling) affecting the left upper and

lower extremities, and a separate episode of decreased vision in the right eye that has

partially resolved. She also endorses fatigue, which she initially attributed to work stress.

**HISTORY OF PRESENT ILLNESS (summarized):**

Ms. Kowalski first presented to this office in June 2023 with complaints of left arm

"falling asleep" - she described episodes lasting minutes to hours. At that time, was

attributed to possible cervical radiculopathy and she was referred for PT. Symptoms improved over approximately 6 weeks and she was lost to follow-up until September 2023

when she returned with continuation of intermittent left-sided numbness now involving

the left leg as well.

In January 2024, patient presented to our associated urgent care with acute decrease in

vision in the right eye described as "like looking through a foggy window" with associated

pain with eye movement. This was initially evaluated by Ophthalmology at Fair Oaks Hospital

(see attached records - note: records may be incomplete, only partial fax received).

Vision improved substantially over 3 weeks but patient notes it has not fully returned to

baseline. She denies diplopia.

Currently (02/28/2024) she complains of persistent mild numbness in the left leg, worse

with heat (hot showers notably exacerbate symptoms), and intermittent tingling in left

hand. Denies bowel/bladder dysfunction at this visit (see prior note 09/2023 - patient

had mentioned some urinary urgency at that time which she did not bring up today).

**PAST MEDICAL HISTORY:**

1. Migraine with aura - diagnosed age 22, managed with sumatriptan PRN
2. Hypothyroidism - on levothyroxine, TSH stable
3. GERD - on omeprazole
4. Seasonal allergic rhinitis - cetirizine PRN
5. Remote h/o vitamin D deficiency - replete 2019, not currently supplementing
6. Anxiety - managed with sertraline, started 2021
7. Appendectomy 2001
8. Cesarean section x2 (2006, 2009)

**FAMILY HISTORY:**

Mother - Type 2 DM, HTN

Father - MI age 58, hyperlipidemia, alive

Maternal aunt - 'some kind of nerve disease' per patient, no further details available

Sister - Hashimoto's thyroiditis

No known family h/o malignancy

**SOCIAL HISTORY:**

Married, 2 children ages 14 and 17. Non-smoker. Rare ETOH (1-2 drinks/month). Denies

illicit drugs. Works full-time as HS biology teacher. Reports significant occupational stress.

Exercises sporadically - previously ran 5K races, has decreased activity due to symptoms.

**CURRENT MEDICATIONS:**

(See attached medication list for full details)

Levothyroxine 75 mcg daily

Omeprazole 20 mg daily

Sertraline 50 mg daily

Sumatriptan 100 mg PRN migraine

Cetirizine 10 mg PRN

Ibuprofen 600 mg PRN (taking ~3x/week for headaches, per patient report)

ALLERGIES: Penicillin (rash), Sulfa (GI intolerance)

**REVIEW OF SYSTEMS:**

Positive: fatigue, sensory disturbance LUE/LLE, h/o visual disturbance RE, headaches

(chronic, consistent with prior migraine pattern per patient), heat intolerance of sx

Negative: fever, weight loss, diplopia (currently), dysphagia, weakness (denies frank

weakness though difficulty with stairs noted), seizure, LOC, cognitive changes (patient

self-reports occasional 'brain fog' but unsure if related)

**PHYSICAL EXAM:**

Vitals: BP 118/74, HR 72, RR 14, Temp 98.2, SpO2 99% RA, Wt 142 lbs, Ht 5'5"

General: Well-appearing, no acute distress

HEENT: Normocephalic, PERRL (ophthalmology note describes afferent pupillary defect in RE during acute episode - resolved per last ophtho f/u)

Neck: Supple, no LAD, no thyromegaly

Cardiovascular: RRR, no murmurs

Pulmonary: CTAB

Abdomen: Soft, NT, no organomegaly

*Neurological exam (limited - performed by IM):*

CN II-XII: grossly intact; visual acuity not formally tested today

Motor: 5/5 throughout - however patient reports subjective weakness L leg with effort

Sensation: Decreased pinprick L arm and L leg compared to right (patchy)

Reflexes: 2+ BLE, 2+ BUE; no Babinski noted but exam limited

Coordination: FNF intact, heel-shin not tested

Gait: Normal appearance in office, patient reports worse with fatigue

**ASSESSMENT AND PLAN:**

Ms. Kowalski is a 45F presenting with episodic and now partially persistent neurological

symptoms including sensory disturbance and history of monocular visual loss with eye pain,

partially resolved. The clinical picture warrants comprehensive neurological evaluation to

determine underlying etiology. Differential considerations include but may not be limited

to inflammatory, demyelinating, or structural processes.

Ordered MRI brain and cervical spine with and without contrast (results attached).

Basic labs ordered (CBC, CMP, TSH, B12, folate, ESR, CRP) - see attached.  
Referral placed to Neurology at JHH for subspecialty evaluation.  
Continue current medications.  
Return to PCP in 4-6 weeks or sooner if symptoms worsen.

Thank you for seeing this patient. Please do not hesitate to contact our office.

Sincerely,

Patricia Ngozi, MD  
Board Certified Internal Medicine  
Riverside Internal Medicine Associates  
NPI: 1982734056

Electronically signed: 02/28/2024 at 14:32  
Dictated and reviewed by attending provider

**\*\* EHR EXPORT - ENCOUNTER NOTE \*\***  
Riverside Internal Medicine Associates

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|                               |                              |
|-------------------------------|------------------------------|
| Patient: KOWALSKI, MARGARET A | DOB: 06/12/1978              |
| Encounter Date: 06/09/2023    | Time: 09:15 AM               |
| Provider: Ngozi, Patricia MD  | Visit: Est. Patient - Office |
| Billing: 99213                | MRN: 04-389-2271             |

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**CHIEF COMPLAINT:**

Left arm numbness / tingling, intermittent x 3 weeks

**SUBJECTIVE:**

Pt is a 44 yo female (DOB confirmed) who presents today with c/o intermittent "pins and needles" sensation in the left arm, predominantly involving the forearm and hand. She

describes episodes lasting 20-45 minutes, occurring 3-4x per week. No precipitating activity noted. Episodes resolve on their own. She denies any associated weakness, neck pain, or trauma. She notes she has been under significant stress at work (end of school year) and wonders if that could be a factor. Denies any similar episodes in past.

No headache with episodes (has chronic migraines but these are distinct per patient).

PMHx: Migraine w/ aura, Hypothyroidism, GERD, Anxiety, Seasonal allergies  
Meds: Levothyroxine 75mcg, Omeprazole 20mg, Sertraline 50mg, Sumatriptan PRN  
Allergies: PCN (rash)

**OBJECTIVE:**

VS: BP 122/78, HR 68, Wt 140 lbs

General: NAD, well-appearing

Neuro (brief): Motor 5/5 bilateral UE, sensation grossly intact to light touch bilaterally

during exam (symptoms not present at time of visit per patient), reflexes 2+ BUE

Musculoskeletal: Neck - full ROM, no tenderness to palpation over C-spine. Spurling's

negative bilaterally. No focal tenderness upper extremities.

**ASSESSMENT/PLAN:**

1. Left upper extremity paresthesias - likely cervical radiculopathy vs. carpal tunnel

vs. thoracic outlet syndrome given unilateral distribution. Low concern for central

process at this time. Will refer to PT for cervical evaluation and nerve gliding.

EMG/NCS if not improved in 6-8 weeks.

2. Hypothyroidism - TSH checked 4/2023: 2.1, within range. Continue Levo 75mcg.
3. Migraine - Sumatriptan refill provided. Discussed limiting ibuprofen due to GERD.
4. Anxiety - Sertraline 50mg, patient reports medication helping. Continue.
5. RTC 8 weeks or sooner if sx worsen or new sx develop.

-- Electronically signed: 06/09/2023 16:02 | Ngozi, Patricia MD --



**\*\* EHR EXPORT - ENCOUNTER NOTE \*\***  
Riverside Internal Medicine Associates

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|                             |                       |
|-----------------------------|-----------------------|
| Patient: KOWALSKI, MARGARET | DOB: 06-12-1978       |
| Date: 09/21/2023 11:30      | Provider: Ngozi, P MD |
| MRN: 04-389-2271            | Billing: 99214        |

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**S:**

*[NOTE: DICTATION BELOW - NOT FULLY EDITED]*

Patient returns today interval several months - states that PT helped somewhat with the

left arm symptoms initially but over the past six to eight weeks symptoms have returned

and in fact are now present in the left leg as well. She describes numbness along the outer

left thigh and sometimes the left foot "goes numb" when she has been sitting for a while.

She is now concerned as the distribution has expanded. She denies any weakness - she says

"my leg works fine" but notes she has been tripping more than usual which she attributes to

"not picking my feet up." She has also noted some urgency with urination over the past

month, occurring maybe 2-3x per week, no dysuria, no hematuria. She initially thought

it was a UTI but urine was clean at urgent care (see outside record - not attached, patient

verbal report only). Denies any visual changes at this time. Fatigue is ongoing, she

attributes to stress + school starting back up. Weight stable.

PMHx as prior - no changes. Added: patient notes maternal aunt was told she had 'something

with her nerves' in her 40s - no further details, aunt passed away from unrelated causes.

**Medications (confirmed at visit):**

Levothyroxine 75 mcg daily

Omeprazole 20 mg daily

Sertraline 50 mg daily

Sumatriptan 100 mg PRN - using 1-2x/month

Cetirizine 10 mg PRN spring/fall

Ibuprofen - patient reports taking regularly for 'tension headaches' ~daily, counseled

**O:**

BP 116/72 HR 74 Wt 141 lbs T 98.0 SpO2 100%

General: alert, oriented, no distress, appears slightly fatigued

Neurological:

Cranial nerves: II-XII intact; fundoscopic exam deferred (no equipment)

Motor: 5/5 BUE and BLE; mild give-way weakness L hip flexor vs. true weakness unclear

Sensation: decreased light touch and pinprick L forearm, L lateral thigh; R side intact

Reflexes: 2+ bilateral; Babinski: flexor bilaterally - exam difficult patient very

ticklish on plantar surface

Gait: observed in hallway - slightly wide-based? Or normal variant, uncertain

Romberg: negative

Musculoskeletal: Spine non-tender to palpation throughout

**A/P:**

1. Progressive left-sided sensory symptoms now involving upper and lower extremity -  
this picture is more concerning than prior visit. Etiology unclear. Differential broad at this point. Will order MRI of brain and cervical spine (ordering today).  
Also ordering: CBC, CMP, TSH, B12, folate, ANA, ESR, CRP. Urgently referred to neurology - patient prefers to wait for MRI results first before neurology appt.
2. Urinary urgency - UA today negative. Likely functional vs related to above, will defer workup pending neuro eval. Oxybutynin discussed but not started at this time.
3. Hypothyroid - stable per recent TSH (will recheck in labs ordered above)
4. Ibuprofen overuse - counseled extensively; switched to acetaminophen PRN
5. Follow up 4 weeks or sooner.

MRI ordered: 09/21/2023 (results received 09/26/2023 - see imaging section)

*Signed: 09/21/2023 @ 13:55 | Ngozi, P MD | Riverside IM*

**FAIR OAKS URGENT CARE CENTER**

12110 Fairfax Towne Center, Fairfax, VA 22033 | (703) 359-7100

**\*\*\* TRANSMITTED VIA FAX - OUTSIDE RECORDS \*\*\***

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|                                     |                                           |             |
|-------------------------------------|-------------------------------------------|-------------|
| Patient Name: Kowalski, Margaret A. | DOB: 06/12/78                             | Age: 45F    |
| Acct #: UC-2024-00814               | Visit Date: 01/08/2024                    | Time: 16:45 |
| Provider: James R. Patel, DO        | Chief Complaint: Eye pain / blurry vision |             |

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**TRIAGE NOTE:**

Patient presents with approximately 3-day history of pain behind the right eye, described as aching and worse with eye movement. She also notes decreased vision in the right eye that she describes as 'cloudy' or 'dim.' No trauma. No discharge. No diplopia. No photophobia. No headache currently (has history of migraines - states this is distinctly different from her migraines). No fever.

Vitals: BP 120/76 HR 80 T 98.1 SpO2 99%

**PHYSICIAN NOTE:**

Seen and evaluated. 45 yo female with above complaints. Visual acuity testing:

OD (right): 20/60 (reduced from pt's reported baseline of 20/20 without correction)

OS (left): 20/20

Color vision: decreased color saturation OD (red cap test - desaturated)

Pupils: Relative afferent pupillary defect (RAPD) noted in right eye

Slit lamp: anterior segment normal OU; no cells in AC

Fundoscopy: Optic disc appears mildly hyperemic OD - subtle; OS normal

EOM: Full but pain with right lateral gaze

Clinical impression: Optic neuritis, right eye - high clinical suspicion given RAPD,

decreased visual acuity and color vision, and pain with eye movement. This is NOT a presentation I feel comfortable managing at the urgent care level.

-- Discussed with patient: referred to ophthalmology same day (Dr. Anita Rao, Fair Oaks --

-- Eye Institute) and also strongly encouraged PCP follow-up. Discussed that optic --

-- neuritis can be associated with systemic conditions; patient advised to discuss with --

-- PCP. Prescribed: nothing at this visit - deferred to ophthalmology. --

NOTE TO RECEIVING PROVIDER: Ophthalmology records from Dr. Rao's office requested; *only partial records received (1 of 3 pages transmitted). See attached - page 3 missing.*

Signed: J. Patel DO 01/08/2024 18:02

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**OPHTHALMOLOGY ADDENDUM (Partial - Dr. A. Rao, Fair Oaks Eye Institute):**

[PAGE 1 OF 3 - REMAINDER NOT RECEIVED]

Patient: Kowalski, M 01/08/2024

Confirmed optic neuritis OD. Recommended IV methylprednisolone course vs. oral prednisone - discussed options. Patient opted for oral course pending further workup.

Prescribed: Prednisone 1mg/kg/day x 10 days (dispensed), followed by 10-day taper.

Ordered: MRI brain w/ and w/o contrast with attention to orbits, MRI orbits w/ fat suppression. Ordered urgent labs (ANA, ANCA, ACE level, RPR, Lyme Ab, CBC, CMP).

RTO ophthalmology 1 week for visual field testing and repeat acuity.

[REMAINDER OF OPHTHALMOLOGY NOTE NOT RECEIVED - FAX INCOMPLETE]

## QUEST DIAGNOSTICS - LABORATORY REPORT

Ordering Provider: Ngozi, Patricia MD | Riverside IM

Requisition #: 2023-QD-8841920      Collected: 09/22/2023      Resulted: 09/23/2023

Patient: KOWALSKI, MARGARET A.      DOB: 06/12/1978      Sex: F      MRN: 04-389-2271

### COMPLETE BLOOD COUNT (CBC WITH DIFF) :

| TEST          | RESULT | UNITS                     | REF RANGE |
|---------------|--------|---------------------------|-----------|
| WBC           | 7.2    | $\times 10^3/\mu\text{L}$ | 4.5-11.0  |
| RBC           | 4.31   | $\times 10^6/\mu\text{L}$ | 3.80-5.10 |
| Hemoglobin    | 13.2   | g/dL                      | 12.0-16.0 |
| Hematocrit    | 39.4   | %                         | 37.0-47.0 |
| MCV           | 91     | fL                        | 80-100    |
| Platelets     | 234    | $\times 10^3/\mu\text{L}$ | 150-400   |
| Neutrophils % | 64     | %                         | 50-70     |
| Lymphocytes % | 28     | %                         | 20-40     |

### COMPREHENSIVE METABOLIC PANEL:

|                 |      |       |           |
|-----------------|------|-------|-----------|
| Sodium          | 139  | mEq/L | 136-145   |
| Potassium       | 4.1  | mEq/L | 3.5-5.0   |
| Creatinine      | 0.72 | mg/dL | 0.50-1.10 |
| BUN             | 14   | mg/dL | 7-25      |
| Glucose         | 88   | mg/dL | 70-99     |
| Total Protein   | 7.1  | g/dL  | 6.4-8.3   |
| Albumin         | 4.2  | g/dL  | 3.5-5.0   |
| ALT             | 22   | U/L   | 7-56      |
| AST             | 19   | U/L   | 10-40     |
| Alk Phosphatase | 68   | U/L   | 44-147    |
| Total Bilirubin | 0.6  | mg/dL | 0.1-1.2   |
| Calcium         | 9.4  | mg/dL | 8.5-10.5  |

### ADDITIONAL LABS:

|             |      |       |           |
|-------------|------|-------|-----------|
| TSH         | 2.4  | mIU/L | 0.40-4.50 |
| Free T4     | 1.1  | ng/dL | 0.8-1.8   |
| Vitamin B12 | 441  | pg/mL | 200-900   |
| Folate      | 12.8 | ng/mL | >3.0      |
| ESR         | 18   | mm/hr | 0-20 (F)  |

|                       |               |       |          |
|-----------------------|---------------|-------|----------|
| CRP, High Sensitivity | 1.2           | mg/L  | <3.0     |
| ANA Screen            | Negative      |       | Negative |
| Vitamin D, 25-OH      | <b>22 (L)</b> | ng/mL | 30-100   |
| Cholesterol, Total    | 198           | mg/dL | <200     |
| HDL                   | 62            | mg/dL | >50      |
| LDL (calc)            | 118           | mg/dL | <130     |
| Triglycerides         | 88            | mg/dL | <150     |

**\*\* REPEAT LABS (Duplicate - EHR export error) \*\***

Labs below ordered 02/28/2024 - partial results available at time of fax:

|                 |               |       |                   |
|-----------------|---------------|-------|-------------------|
| TSH             | 2.1           | mIU/L | 0.40-4.50         |
| CBC             | See prior     | --    | (pending recheck) |
| CMP             | See prior     | --    | (pending recheck) |
| Vitamin D 25-OH | <b>19 (L)</b> | ng/mL | 30-100            |

*NOTE: RPR, Lyme Ab, ANCA, ACE level ordered by Ophthalmology (01/08/2024) - results not available in our system. Patient was asked to request from Rao's office.*

## INOVA RADIOLOGY SERVICES

Inova Fair Oaks Hospital | 3600 Joseph Siewick Dr, Fairfax VA 22033

### Radiology Report - MRI BRAIN WITH AND WITHOUT CONTRAST

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Patient: KOWALSKI, MARGARET ANN      MRN (Inova): 8-2291-0034  
DOB: 06/12/1978      Sex: F      Age: 45      Ordering MD: Ngozi, Patricia MD  
Exam Date: 09/25/2023      Exam Time: 14:20      Read Date: 09/26/2023  
Accession #: INV-RAD-9923-88204      Priority: Routine

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#### CLINICAL INDICATION:

45F with progressive left-sided sensory symptoms. R/O structural or inflammatory lesion.

#### TECHNIQUE:

MRI of the brain performed at 3.0 Tesla. Sequences: axial T1, T2, FLAIR, DWI, and sagittal T1. Post-contrast imaging with gadolinium (Gadavist 0.1 mmol/kg IV) in axial and coronal planes.

COMPARISON: None available.

#### FINDINGS:

Brain parenchyma: There are multiple ovoid and elongated T2/FLAIR hyperintense foci within the supratentorial white matter, predominantly periventricular in distribution, with the long axis perpendicular to the lateral ventricles. The lesions measure between 4mm and 14mm in greatest dimension. Several demonstrate callososeptal involvement. No evidence of cortical lesion.

Post-contrast: One lesion in the right periventricular white matter demonstrates ring-like peripheral enhancement, measuring approximately 9mm. Remaining lesions do not demonstrate enhancement. No leptomeningeal enhancement. No mass effect.

Posterior fossa: A single 5mm T2 hyperintense focus within the left middle cerebellar peduncle is noted. No enhancement.

Corpus callosum: Subtle thinning of the inferior body. Several callososeptal lesions noted ("Dawson fingers" morphology, see above).

Ventricles: Normal in size and configuration.  
Midline: No shift.  
Extra-axial spaces: Unremarkable.

Vascular flow voids: Maintained.

Orbits: Partially imaged; no focal signal abnormality in the visualized optic nerves

on this non-dedicated study. (Note: dedicated orbital imaging not obtained.)

Sinuses: Mild mucosal thickening maxillary sinuses bilaterally, incidental.

**IMPRESSION:**

1. Multiple white matter T2/FLAIR hyperintensities with periventricular predominance,

callososeptal involvement, and perpendicular orientation to the ventricular surface.

One lesion demonstrates active enhancement. The imaging pattern is most consistent

with a demyelinating process. Clinical correlation and subspecialty consultation recommended.

2. Single posterior fossa lesion in the left middle cerebellar peduncle.

3. No acute hemorrhage, infarction, or mass lesion identified.

*Electronically signed: Michael T. Ogunwale, MD | Neuroradiology*

*09/26/2023 @ 11:42 AM*

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**MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST**

Same patient | Exam Date: 09/25/2023 | Accession #: INV-RAD-9923-88205

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TECHNIQUE: MRI C-spine 3.0T, sagittal T1/T2/STIR, axial T2, post-contrast sagittal T1.

**FINDINGS:**

Alignment: Maintained.

Cord signal: A focal T2 hyperintense lesion is identified within the cervical spinal cord

at the C4-C5 level, involving less than half the cross-sectional area of the cord, measuring approximately 8mm craniocaudally. No cord expansion. No enhancement.

Intervertebral discs: Mild disc bulge at C5-C6 without significant neural foraminal or canal stenosis. Remaining levels unremarkable.

Vertebral bodies: Normal height and signal.

Paraspinal soft tissues: Unremarkable.

**IMPRESSION:**

1. Focal T2 hyperintense intramedullary lesion at C4-C5. In the context of the brain

MRI findings above and the clinical history provided, this is consistent with a demyelinating cord lesion.

2. Mild C5-C6 disc disease without significant stenosis.



*Signed: M. Ogunwale MD | 09/26/2023 @ 11:58*

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**\*\* EHR MEDICATION LIST EXPORT \*\***

Riverside Internal Medicine Associates | Patient: Kowalski, Margaret A.

Generated: 02/28/2024 15:01 | MRN: 04-389-2271

**\*\*\* THIS LIST MAY NOT REFLECT OTC MEDICATIONS OR MEDICATIONS PRESCRIBED BY \*\*\***

**\*\*\* OUTSIDE PROVIDERS - VERIFY WITH PATIENT AT TIME OF VISIT \*\*\***

| MEDICATION         | DOSE / SIG                                                                       | ROUTE | PRESCRIBER | STATUS                  |
|--------------------|----------------------------------------------------------------------------------|-------|------------|-------------------------|
| Levothyroxine      | 75 mcg qAM                                                                       | PO    | Ngozi      | Active                  |
| Omeprazole         | 20 mg daily                                                                      | PO    | Ngozi      | Active                  |
| Sertraline         | 50 mg daily                                                                      | PO    | Ngozi      | Active                  |
| Sumatriptan        | 100 mg PRN<br>migraine; max<br>2/24hr                                            | PO    | Ngozi      | Active                  |
| Cetirizine         | 10 mg PRN<br>allergy sx                                                          | PO    | Ngozi      | Active                  |
| Acetaminophen      | 500 mg q6h PRN<br>pain/HA (max<br>3g/day)                                        | PO    | Ngozi      | Active                  |
| Prednisone         | Taper - started<br>01/08/24<br>(completed)                                       | PO    | Rao (ext)  | INACTIVE -<br>completed |
| Ibuprofen 600 mg   | PRN - PATIENT<br>COUNSELED TO<br>STOP                                            | PO    | Self/OTC   | Discouraged             |
| Vitamin D3 (OTC)   | Patient not<br>currently taking<br>(counseled to<br>start 1000-2000<br>IU daily) | PO    | Discussed  | Recommended             |
| Multivitamin (OTC) | 1 tab daily -<br>patient self-<br>report                                         | PO    | Self       | Active<br>(OTC)         |

**ALLERGIES (confirmed 02/28/2024):**

Penicillin - Rash (moderate)

Sulfa drugs - GI intolerance (moderate)

NKDA otherwise per patient report

**FAIR OAKS HOSPITAL - EMERGENCY DEPARTMENT**

3600 Joseph Siewick Drive, Fairfax, VA 22033

**ED ENCOUNTER SUMMARY / DISCHARGE INSTRUCTIONS**

[OUTSIDE RECORDS - FAX RECEIVED 03/12/2024]

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Patient: Margaret Kowalski DOB: 06/12/1978 MRN (INOVA): 8-2291-0034

ED Visit Date: 01/08/2024 Arrival: 1623 Discharge: 1914

Attending: Priya Mehta, MD (Emergency Medicine)

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**REASON FOR VISIT:**

Right eye pain and decreased vision - acute onset x 3 days

**HISTORY (from ED intake and nurse triage):**

45-year-old female with history of migraines presenting with right eye pain and blurred

vision. Pain described as aching, behind the eye, worse with eye movement. Vision described

as 'foggy.' No trauma. No known sick contacts. No recent travel (overseas trip in 2022 -

patient volunteered this information though unclear relevance). Denies fever, rash, joint

pain. Has existing history of left arm and leg numbness (per patient) - not the reason

for today's visit per patient. Takes levothyroxine for thyroid, sertraline for anxiety.

Allergies: penicillin, sulfa.

NOTE FROM TRIAGE RN: Patient mentioned that she saw her PCP recently for 'neurological

symptoms' and had an MRI done. She does not have results with her today.

**ED ASSESSMENT / COURSE:**

Patient evaluated by ED attending and ophthalmology consulted (see separate urgent care

note - patient first presented to urgent care prior to ED, unclear why visit duplicated),

Ophthalmology confirmed optic neuritis right eye. Disposition: discharge home with ophthalmology follow-up and primary care follow-up. Steroid course prescribed by ophthalmology per their assessment.

**DISCHARGE DIAGNOSIS:**

1. Right eye optic neuritis - etiology undetermined
2. Existing neurological complaints per patient history (not worked up in this ED visit)

**DISCHARGE INSTRUCTIONS:**

- Follow up with ophthalmology Dr. Rao within 1 week (appointment made: 01/15/2024)

- Follow up with PCP within 1-2 weeks
- Take steroid medication as prescribed
- Return to ED immediately if: complete vision loss, new neurological symptoms, difficulty speaking or walking
- Patient verbalized understanding and was discharged ambulatory in good condition

*Electronically signed: P. Mehta MD | 01/08/2024 19:01*

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**OPHTHALMOLOGY FOLLOW-UP NOTE (Dr. A. Rao - 01/15/2024) [PARTIAL]:**

*[NOTE: Only first page of 2 received. Second page not transmitted.]*

Patient: Kowalski M 01/15/2024 Follow-up optic neuritis OD  
Visual acuity today: OD 20/30 (improved from 20/60 at initial visit). Subjective improvement in color vision. RAPD diminished but persists mildly. Patient completed oral prednisone course. VEP ordered (visual evoked potentials) - to be performed at [---TEXT CUT OFF - BOTTOM OF PAGE---]

*[PAGE 2 NOT RECEIVED]*

## NOVA PHYSICAL THERAPY & REHABILITATION

13921 Lee Jackson Memorial Hwy | Chantilly, VA 20151 | (703) 263-4400

### PHYSICAL THERAPY EVALUATION NOTE

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Patient: KOWALSKI, MARGARET      DOB: 06/12/78      Date: 07/03/2023  
Referral Source: Dr. Ngozi / Riverside IM      Dx: Cervical radiculopathy (M54.12)  
Therapist: Kevin S. Barrington, PT, DPT

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#### REASON FOR REFERRAL:

45 yo female referred for evaluation and treatment of left upper extremity paresthesias and possible cervical radiculopathy per referring MD. Patient reports intermittent numbness and tingling in left arm, sometimes involving forearm and hand. Onset approximately 1 month ago. Symptoms are intermittent - not present at time of this eval.

#### SUBJECTIVE:

Patient reports episodes of 'pins and needles' in left arm, duration variable (minutes to over an hour). Does not consistently correlate to neck position or activity. She has not noted exacerbation with Valsalva. Denies radicular pain per se - more 'buzzing' quality. No hand weakness or grip difficulty. No bowel/bladder symptoms. No gait disturbance at this time. Works as a teacher, stands a lot during workday.

Numeric Pain Rating: 2/10 (intermittent)

Functional limitation: Mild - occasional concern about arm sensation while driving

#### OBJECTIVE FINDINGS:

Posture: Forward head posture, mild rounded shoulders bilaterally  
Cervical ROM: WNL all planes with mild discomfort at end-range left rotation  
Spurling's: Negative bilaterally  
ULTT: Equivocal left median nerve tension test - mild paresthesia reproduction  
Grip strength (dynamometer): R 52 lbs, L 49 lbs (within functional limits)  
Sensation: Patient reports intermittent numbness but exam unable to confirm - grossly intact to light touch at time of evaluation  
Neurological screen: DTRs 2+ BUE; no UMN signs elicited  
Palpation: Mild tenderness bilateral upper trapezius and levator scapulae

#### ASSESSMENT:

Presentation is somewhat atypical for classic cervical radiculopathy given negative Spurling's and lack of consistent positional component. Possible thoracic outlet component vs. early peripheral neuropathy vs. central process not ruled out. Proceeding with conservative cervical/TOS-directed PT per referral while symptoms

are mild. If no improvement or symptom progression, will communicate with referring MD.

**PLAN:**

2x/week x 6 weeks. Cervical stabilization, postural retraining, nerve gliding exercises,  
thoracic mobility. Home exercise program provided.

**DISCHARGE SUMMARY (08/11/2023 - attached to this note):**

Patient completed 6 sessions (attended 5 of 6 scheduled). Reports ~50% reduction in frequency of paresthesia episodes. Cervical ROM improved. Patient felt exercises helpful.

Discharged to HEP. RTC PT PRN. Communication sent to Dr. Ngozi 08/14/2023.

[Note in chart: patient called 09/05/2023 to report symptoms returning - advised to follow up with PCP per our communication]

*K. Barrington, PT, DPT* | *NPI: 1234567890*

**\*\* EHR EXPORT - ENCOUNTER NOTE \*\***  
Riverside Internal Medicine Associates

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Patient: KOWALSKI, MARGARET      DOB: 06/12/1978      MRN: 04-389-2271  
Date of Service: 10/19/2023      Provider: Ngozi, Patricia MD  
Visit Type: Office - Established      Billing: 99213

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**ADDENDUM / FOLLOW-UP NOTE:**

Patient called 10/18 requesting to discuss MRI results. Phone encounter converted to in-person visit per patient request. Patient accompanied by husband today.

**INTERIM HISTORY:**

Since last visit (09/21/23), left-sided sensory symptoms have been persistent with some fluctuation. She notes improved symptoms when she was on vacation (cooler weather, less stress per patient). Urinary urgency has improved somewhat. No new visual complaints. No falls. No new weakness. She has been researching her symptoms online and is 'worried about something serious.' She specifically mentioned 'lesions on her brain' which she had read about.

**DISCUSSION OF MRI RESULTS:**

I reviewed the MRI brain and cervical spine results (report dated 09/26/2023) with the patient and her husband. I explained that the radiologist identified several abnormal areas in the white matter of the brain and one area in the cervical spinal cord. I explained that these findings required subspecialty evaluation to determine their significance and cause. I was deliberate in not providing a specific diagnosis at this visit as I felt this required proper subspecialty workup and was beyond the scope of this IM visit. Patient and husband were understandably anxious.

I explained that a neurology referral had been placed and emphasized the importance of prompt follow-up. Patient asked about the possibility of 'a tumor' - I explained the characteristics of the imaging were not consistent with that but specialist review is warranted.

**PLAN:**

1. Neurology referral - expedited if possible. Patient will call scheduling directly.
2. Labs ordered 09/22 reviewed (see prior) - notably Vit D low at 22, recommending supplementation. ANA negative. ESR/CRP not significantly elevated.

3. Continue all current medications.
4. Patient counseled regarding symptom monitoring - instructed to return or call if new or worsening neurological symptoms, vision changes, or falls.
5. Return to PCP 4 weeks.

Patient left handout regarding vitamin D supplementation and was given written copy of MRI report per request.

Time spent: 25 minutes (>50% counseling/coordination of care)

-- Signed: Ngozi, Patricia MD | 10/19/2023 @ 15:14 --

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**[SCANNED PAGE - HANDWRITTEN TELEPHONE NOTE - PARTIAL]**

Date: ~11/2023 (exact date illegible)

Pt: Kowalski M | Re: neurology appt status

Called to check on referral - states Hopkins had 4-month wait, will try to be seen sooner if cancellation. In meantime PCP office advised pt to call with any changes.

New eye symptom started January - see separate UC/ED records above [ref added editorially]



## VISUAL EVOKED POTENTIAL REPORT

Fair Oaks Eye Institute | 3600 Joseph Siewick Dr Suite 400, Fairfax VA  
[OUTSIDE RECORDS]

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Patient: Kowalski, M.A. DOB: 06/12/1978 Date of Test: 01/29/2024  
Ordering: Rao, Anita MD (Ophthalmology) Tech: CMF

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**PROCEDURE: Pattern visual evoked potentials (P-VEP), binocular and monocular**

### RESULTS:

Right eye (OD): P100 latency 118 ms (normal <110 ms) - PROLONGED  
Amplitude: 4.2 uV (low normal)  
Left eye (OS): P100 latency 104 ms (normal) - NORMAL  
Amplitude: 8.1 uV (normal)

INTERPRETATION: Prolonged P100 latency in the right eye is consistent with delayed conduction in the right optic nerve. This finding, in the context of the clinical history provided (recent optic neuritis OD), is consistent with demyelination-related conduction delay. Left eye within normal limits.

*Interpretation by: B. Chen, PhD (Clinical Neurophysiology) 01/30/2024*

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### INSURANCE AUTHORIZATION DOCUMENTATION

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Auth Request Date: 03/01/2024 Status: PENDING as of 03/14/2024  
Service Requested: Outpatient Neurology Consultation (CPT 99245)  
Insurance: BCBS FEP Auth #: PENDING  
Note: Prior auth may be required. Office staff working on auth as of date of fax.  
Call (571) 240-8800 ext 3 for auth status updates.

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### ADDITIONAL NOTE FROM REFERRING OFFICE:

Please note: Patient was also briefly seen at our associated urgent care on 11/30/2023 for an unrelated complaint (URI/sinusitis - treated with amoxicillin NB patient has PCN allergy - this was a DOCUMENTATION ERROR, patient was NOT given amoxicillin, treated with azithromycin per allergy note - see corrected record in chart flag). This visit is included in EHR export by error and is NOT relevant to this referral.

Also: Patient reports her sister was recently diagnosed with Hashimoto's. Maternal aunt's 'nerve disease' - patient attempted to get records. Aunt passed 2018, records not located.

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**END OF REFERRAL PACKET - Pages 1-18**

Transmitted by: L. Hoffman (Medical Records) | Riverside IM Associates

Fax transmission completed: 03/14/2024 10:52 AM